

PREVENT VERTICAL TRANSMISSION OF HIV

Assess and manage the newborn exposed to HIV

- Do HIV positive mother's viral load at delivery and HIV PCR test on her baby as soon after birth as possible (within 48 hours). Place barcodes on discharge form and RtHB.
- If abandoned baby, do rapid HIV test¹ and HIV PCR test on baby. If < 72 hours since delivery, manage as high risk formula feeding baby below.

Start post-exposure prophylaxis (PEP) as soon as possible, ideally within 1 hour of birth

- Give baby **zidovudine (AZT)** 12 hourly (see dosing table below) and give **nevirapine (NVP)** once daily (see dosing table below). Give supply for 6 weeks and advise carer to bring all medication to next visit.
- Advise to return for baby's HIV PCR and mother's viral load results in 3-6 days.

At 3-6 day postnatal visit, check results of baby's HIV PCR and mother's viral load and manage according to results:

If results not available, continue AZT and NVP and follow-up after 1 week. If no HIV PCR done, do at this visit and follow-up after 1 week.

Baby's HIV PCR negative				Baby's HIV PCR positive	Baby's HIV PCR indeterminate
Mother's VL < 50 at delivery		Mother's VL ≥ 50 or unknown at delivery			
Low risk		Higher risk			
• Stop AZT². • Give NVP daily for 6 weeks (see table). • If breastfeeding: repeat mother's VL 6 monthly.		• Manage mother's unsuppressed VL ↗ 166. • If mother's VL ≥ 1000, discuss need for HIV resistance test for mother and baby with HIV expert/hotline ↗ 178.			
		Breastfeeding ³		Formula feeding	
		• Give AZT 12 hourly for 6 weeks (see dosing table below) <i>and</i> • Give NVP daily for at least 12 weeks (see dosing table below) • Stop NVP only once mother's VL < 50 or 4 weeks after final breastfeed. • If mother on TLD2 or 3rd line ART for ≥ 3 months <i>and</i> VL ≥ 1000, alert to risks of breastfeeding, discuss changing to formula feeding and refer to nutritional therapeutic programme (NTP). Discuss with HIV expert/hotline ↗ 178.	Give AZT (12 hourly) <i>and</i> NVP (daily) for 6 weeks (see dosing tables below).		
			• Send 2nd HIV PCR test and refer to doctor to change to ART. • Advise mother to breastfeed for at least 2 years. • If formula feeding, consider feasibility of re-establishing breastfeeding.	• Continue HIV PEP according to mother's delivery VL result (see adjacent). • Do HIV PCR test and HIV viral load, review child and check results within 3 days.	

- Repeat baby's HIV test at 10 weeks, 6 months, 18 months, 6 weeks after final breastfeed or nevirapine stopped (if given ≥ 12 weeks) or any time if baby unwell.
- If mother's VL ≥ 50 at any time during breastfeeding after NVP and/or AZT stopped or mother tests HIV positive for the first time during breastfeeding: repeat/do child's HIV test at that visit, restart/start NVP and AZT (see above), and reassess once child's HIV test result available.

Nevirapine syrup (10mg/mL)		
Age	Current Weight	Once daily dose
Birth to 6 weeks	2-2.49kg ⁴	1mL (10mg) daily
	≥ 2.5kg	1.5mL (15mg) daily
6 weeks to 6 months		2mL (20mg) daily
6 to 9 months		3mL (30mg) daily
≥ 9 months		4mL (40mg) daily

Zidovudine syrup (10mg/mL)		
Age	Current Weight	12 hourly dose
Birth to 6 weeks	2-2.49kg ⁴	1mL (10mg) 12 hourly
	≥ 2.5kg	1.5mL (15mg) 12 hourly
6 weeks to 6 months		6mL (60mg) 12 hourly
≥ 6 months		Dose 12 hourly according to weight.

¹An HIV rapid test shows whether baby was exposed to HIV, but cannot determine whether baby is infected with HIV. An HIV PCR test determines if baby is infected with HIV. ²Return unused AZT to pharmacy to be discarded. ³A breastfed baby has breastfed in the past 7 days or is mixed feeding. ⁴If weight < 2kg, discuss medication options with HIV expert/hotline ↗ 178.